

The present attitude of parish councils towards medical officers is due to a supposition based on "judge-made law" and the "opinions" of Local Government Board law officials. Neither the one nor the other, I respectfully submit, have the right to assume legislation on behalf of parish councils to the disadvantage of the medical officers, and I firmly believe that if the utmost publicity were given to three or four more cases similar to Dr. Macara's, an end would soon be put to the present very unsatisfactory position of Highland parish doctors.

The position is as follows: The Local Government Board has *statutory* power to dismiss a medical officer, but, so it says, cannot retain him in office. Parish councils, on the other hand, have arrogated to themselves the power to dismiss the doctor, but they cannot retain him in office. Thus both authorities can dismiss a medical officer, but neither of them can retain him in office. It seems to me that the only authority who can retain the doctor in office is himself if he has the pluck to do so, only he would need to possess an immaculate record, and powerful legal and financial support, because he cannot fall back on the rate-payers' pockets to defend himself as Mr. Bumble can.

The Local Government Board reserves to itself the right to approve of the appointment of a medical officer. This was denied to me by an official of the Local Government Board; yet, strange to say, that authority some years ago refused to sanction the appointment of a legally-qualified and registered medical practitioner. In this case it so happened that the Board was justified; but still, it proves my contention that it has that power over parish councils. It must approve of the salary given to the doctors. It formulates stringent rules for his guidance; and finally, it, and it only, possesses *statutory* power to dismiss him. Notwithstanding this, the Local Government Board says that it cannot help us, so it seems little use appealing to the Board.

At the risk of appearing revolutionary, I would again advise all Highland parish doctors to join both medical defence unions, and, if threatened with arbitrary dismissal, appeal formally to the Local Government Board, intimating at the same time that legal proceedings will be taken against the parish council if necessary.

I re-echo Dr. Bruce's advice, if "he is to die, let him die hard." He dies in a good cause, namely, an attempt to emancipate his professional brethren from Bumbledom.—I am, etc.,

May 3rd.

ONE OF THEM.

MR. ARNOLD LUPTON AND VACCINATION.

SIR,—In the BRITISH MEDICAL JOURNAL of April 21st you honoured me by some references to a calculation which I put forward to the effect that 100,000 people were annually killed in this country by vaccination. In reply to your comments, I may say that I have not pledged myself, and do not pledge myself, to the accuracy of that calculation; but this I do say, that I should have no right to circulate such a calculation unless I believed that there was an authoritative basis for the figures. It may be that a careful inquiry would show that the actual number of deaths caused every year by vaccination in this country was less than 100,000. Suppose, for the sake of argument, that the provaccinating fraternity, analysing the figures in the method suggested by your article, reduced the number of deaths caused annually in this country by vaccination to 10,000 people, still that figure would be sufficiently startling to make ordinary people pause and consider whether or not they desire to pay for such a slaughter of the innocents.

But now as to the calculation which I did put forward. I have before me the Government paper. It was printed by order of the House of Commons and dated August 14th, 1877. It is headed, "Vaccination Mortality." That heading was put by the Registrar-General, not by me. This return gives the deaths over a period of 28 years from 15 causes, including small-pox. In the 7 years ending with 1853 the average number of deaths from these 15 causes per annum was, in England and Wales alone, about 138,000.

In 1854 vaccination had become compulsory, and from that date the deaths from these same diseases (or causes) went up and up till, in 1875, the number of deaths was, in one year, 217,700. This showed an increase in the deaths from these causes of nearly 80,000 a year, following on

compulsory vaccination, in England and Wales alone. Is it not likely that a similar increase took place in Scotland and Ireland? If it did, then there were more than 20,000 additional deaths in those countries, making the total of 100,000 deaths given in the calculation referred to.

You say 100,000 deaths equal 1 in 5 of all deaths, but the calculation refers to the United Kingdom, and in 1877 the total of deaths was not very far short of 700,000, so that the deaths due to vaccination (according to the calculation) were 1 in 7 of all deaths.

The figure of 100,000 deaths requires, of course, correcting for increase of population, and this reduces the deaths, possibly due to vaccination, to 83,000. But, on the other hand, there ought to have been a decline in the death-rate from these fifteen causes due to sanitation; unfortunately the effects of sanitation were counteracted by vaccination, so these Government figures indicate an increase in deaths of 100,000 a year due to vaccination.

I think I have thus shown very clearly that whatever may be the real number of deaths by vaccination, the calculation which I have put forward is founded upon authoritative Government figures, and is such that any candidate for Parliament or member of Parliament who believes either compulsory vaccination or vaccination in itself to be wrong may justly put forward in support of his case.

You state that some of these deaths from the fifteen diseases could not be due to vaccination, but there I respectfully differ from you. No medical man who has any knowledge at all of the meaning and theory of vaccination will deny that if it is properly done it causes an illness; neither will he, if he is an honest and competent witness of experience, deny that, under certain conditions and in certain children, that illness may be serious. Every competent and honourable man will also admit that if a person is suffering from an illness by which they are weakened they are more liable than a person in health to die from some other ailment which may attack them in their weakened condition; and every competent and honourable medical man will also admit that at the time of death the patient not infrequently suffers from a combination of ailments. That being so, it follows that any baby or adult person who is made ill by vaccination may die of bronchitis, diarrhoea, or almost any other ailment in the Registrar-General's vocabulary, and the death may be really due to vaccination.

With regard to syphilis, it was shown by the eminent Dr. Creighton that there was a great increase in deaths from infantile syphilis following upon compulsory vaccination. It was also shown by that eminent doctor and by others that there is a striking similarity between some of the effects of cow-pox (vaccination) and syphilis, and that the cases of the production of this terrible disease were not so rare as you would suggest.

A study of some of the writings of learned medical advocates of vaccination will show that its effects often endure for many years, and, indeed, I think no competent medical man will deny that the effects of some illness in childhood may endure for the whole of a person's life; it may result in some weakened condition of the nerves, or in some predisposition of other parts of the body which may make it less able to resist, even in middle or old age, such a disease as cancer. There are dentists who say that the growth of teeth in babies is seriously injured by vaccination. I may say that in Germany babies are exempted from compulsory vaccination up to the age of 12 months; and I am happy to think that the President of the Local Government Board, in his speech in the debate on April 25th, suggested the raising of the exemption limit to 5 years.

It was certainly very surprising, if the use of humanized lymph was so harmless as you would suggest, that in the Act of 1898 the use of calf lymph only should be made compulsory, thereby at one stroke upsetting the practice defended by the medical fraternity for a hundred years. I do not know if any one uses the humanized lymph now, but I know medical men who say that on no account would they use it. Surely we cannot now rely on the authority of the medical profession, when its established practice of a hundred years is thus rudely set aside?

I and most other antivaccinators condemned the use of humanized lymph, but I never for one moment suggested—and I do not know that any other antivaccinator has done so—that calf lymph was any better; indeed the

evidence of many medical men (including Jenner) who have studied the question during the last hundred and ten years is that calf lymph is much more dangerous than humanized lymph.

The terrible cases that I have seen myself and others that I know of prove this calf lymph to be a horrible and dangerous thing to put into the blood of any human being, and I trust that the time is not far distant when the honourable profession you represent and the great medical literature of this country will discard the use of policemen and police magistrates for the enforcement of an eighteenth-century nostrum and rely upon modern science in their combat with disease.—I am, etc.,

House of Commons, April 30th.

ARNOLD LUPTON.

INFLUENCE OF X RAYS UPON TREATMENT OF FRACTURES AND DISLOCATIONS.

SIR,—I am much interested—as all medical radiographers must be—in Mr. Lynn Thomas's paper and illustrations in the *BRITISH MEDICAL JOURNAL* of May 5th, p. 1034.

But are not the appearances he describes simply the perspective appearances which these same objects would present to an eye in the position of the focus tube in each case, supposing the objects portrayed were correspondingly visually transparent?

The essential ambiguity of an *x*-ray representation consists in the fact that, owing to the centrifugal nature of the projecting agent, bodies opaque to *x* rays eclipse everything both beyond them and between them and the focus tube, whereas transparent bodies come out on the plate indifferently one through another, without any blurring by refraction. Hence without stereoscopic methods we are restricted to perspective considerations alone in determining the relative distances of the parts depicted.

Yet from perspective principles alone it is possible sometimes to draw definite and reliable conclusions. Thus in Mr. Thomas's figure 2A and B, from inspection it is almost certain that, at the times the radiographs were taken, in each case the lower fractured outline of the head of the humerus was further from the plate than the upper broken edge of the shaft. Nor is any personal examination of the patient necessary to determine the fact.

If the side (right or left) and the position of the patient in reference to the focus tube and plate be known, further inferences of a practical kind could be safely drawn, not otherwise obtainable than by means of the radiograph to start with.—I am, etc.,

Bristol, May 5th.

W. COTTON, M.D.

THE HEAD QUARTERS OF THE ASSOCIATION.

SIR,—As the proposer of the resolution for extending the Library hours which was carried unanimously at a meeting of the City Division, I am indeed sorry to learn that the Library Committee does not see its way to keep open the room till 11 p.m.

I did not propose this resolution until I had taken the opinion of a large number of members, who without exception were in favour of the Library being kept open until a later hour. I would ask, Have the Library Committee considered thoroughly the needs of those who, on account of their work, cannot use the Library during the afternoon? I am afraid not.

I see by the accounts published in the *SUPPLEMENT* of April 28th that £109 16s. has been spent during 1905 in purchasing books, and that it has cost the Library Committee £123 2s. 2d. to do this.

May I take this occasion to express my thanks to the courteous and painstaking Librarian for the kindly help which he has always given me?

I am pleased to see that Dr. Broadbent has taken up the question of the rebuilding of 429, Strand, and trust that we may hear from him again more fully on this matter.

At the same meeting of the City Division, at which I proposed that the Library be kept open till 11 p.m., I proposed, and this resolution was also carried unanimously:

That better accommodation be provided at head quarters for the social intercourse of members, and especially provincial members.

I also propounded at that meeting a scheme by which it might be carried out economically, which at present would be too lengthy for your paper. I have during the past six months visited a large number of members, in order to sound them with regard to my proposals, and I cannot remember a single instance where they were not favourably entertained. I can hardly agree with the Medical Secretary as to the awakening of activity in the Divisions, for I have found it otherwise. To me the lukewarmness and apathy is disheartening. I believe I am right in saying that it is often difficult to get a quorum at the meetings of many of the Divisions. Just lately the City Division had to reduce the number necessary to form a quorum from 12 to 8.

The question has been often asked, What do we get for our 25s. a year? the answer to which is, the *JOURNAL*. That is all; nothing more. Surely it is time something was done to bring the members more together socially, so that we may obtain that goodwill and unity so needful in our profession—something done to render an Association such as ours a power, not a laughing-stock, as it was made in the case of Mr. Troutbeck the coroner.—I am, etc.,

London, N., May 2nd.

STUART HALLOWS.

FISH-EATING AND LEPROSY IN THE ORKNEYS AND IN SHETLAND.

SIR,—Dr. Skae says: "Not a single case [of leprosy] has occurred in the course of over a century."

I recollect seeing a case of what was thought to be tuberculous leprosy in the Edinburgh Infirmary in 1854 or 1855. It was in a lad who came, I believe, from either Unst or Yell.—I am, etc.,

Bradford-on-Avon, May 5th.

JOHN BEDDOE.

PUBLIC HEALTH AND POOR-LAW MEDICAL SERVICES.

THE PREVENTION OF CONSUMPTION.

FOLLOWING the receipt of the circular issued a few months ago by the National Association for the Prevention of Consumption, the Borough Council of Rotherham called for a report from the medical officer of health as to the provisions available in the borough for combating tuberculosis. In this report, which was recently considered and adopted by the Public Health Committee. Dr. Alfred Robinson first expresses his entire agreement with the objects and methods of the Association as set forth in the circular. He then points out how the preventive measures there suggested may be made applicable to the conditions prevailing in Rotherham, where one person out of every five between the ages of 15 and 60 dies of pulmonary consumption. The requirements include preventive measures as regards diseased meat, infected milk, and the dissemination of infection by phthisical persons.

Two requirements with regard to meat are insisted upon—meat inspection, with the destruction of diseased flesh, and the abolition of private slaughterhouses in order to render inspection more efficient. Dr. Robinson recommends that the wide powers with respect to the infection by milk which are given by the Rotherham Corporation Act (1904) should be more fully utilized. He is not in favour of the municipal supply of milk, but considers municipal control to be of far greater importance.

As regards the dissemination of tuberculosis by phthisical persons, he urges the enforcement of by-laws against spitting; the fuller utilization of the powers already possessed by the council for the enforcement of proper ventilation of factories and workshops; the voluntary notification of phthisis, and the education of patients by means of visits and leaflets, the disinfection of rooms, the supply of spit bottles and disinfectants, and provision for the isolation and sanatorium treatment of patients. Dr. Robinson recommends the utilization of a special ward in the workhouse which has already been in use for some time, and of vacant wards in the new isolation hospital. He also suggests that the new small-pox hospital might be built to accommodate phthisical patients when there is no small-pox in the district.

The prompt and practical consideration by the Rotherham Borough Council of the circular of the National Association for the Prevention of Consumption indicates that the issue of this circular may be productive of a widespread activity in preventive efforts throughout the country.

PUBLIC VACCINATION FEES.

A CIRCULAR letter recently issued by the Leigh Board of Guardians has not met with the entire approval of those Poor-law authorities to whom it was addressed. The communication urged that a united effort should be made to bring about a diminution in the cost of vaccination by a reduction of the